

D445

Intermediate Nursing Skills

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 67-year-old retired firefighter receiving IV fluids at 100 mL/hr through a forearm catheter, placed two days ago by the night nurse, says the arm feels tight. The site is swollen, pale, and cool to touch, and the pump alarms for occlusion. What is happening?

- A. A pump problem; replace the tubing.
- B. A normal finding after two days; slow the rate.
- C. Infiltration; stop the infusion and remove the catheter.
- D. Phlebitis; apply a warm compress and keep the IV running.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

An IV site is red, warm, and tender, with a firm cord along the vein. Which complication is this?

- A. Circulatory overload
- B. Infiltration
- C. Air embolism
- D. Phlebitis

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

On a rural unit where pumps are all in use, a patient who works as a school custodian needs 1,000 mL of normal saline over 8 hours. The tubing drop factor is 15 gtt/mL. The bag was hung at 0800. How many drops per minute?

- A. 8 gtt/min
- B. 125 gtt/min
- C. 31 gtt/min
- D. 1,875 gtt/min

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

A gravity IV is running at 25 gtt/min through tubing with a drop factor of 10 gtt/mL. What is the hourly rate?

- A.** 25 mL/hr
- B.** 15,000 mL/hr
- C.** 2.5 mL/hr
- D.** 150 mL/hr

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

Ten minutes into a unit of packed red blood cells, a 58-year-old bank manager, whose brother donated blood last week, reports chills and low back pain. His temperature rose 1.5 °C. What should the nurse do first?

- A.** Stop the transfusion.
- B.** Notify the blood bank that the brother's blood may be the cause.
- C.** Slow the transfusion and recheck vital signs in 15 minutes.
- D.** Give acetaminophen and continue.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Which IV solution may be run through the same line as packed red blood cells?

- A.** 0.9% sodium chloride
- B.** Any isotonic fluid
- C.** 5% dextrose in water
- D.** Lactated Ringer's

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A patient with a tracheostomy, who was a long-distance runner and communicates with a whiteboard, needs suctioning. His oxygen saturation is 94%. How should the nurse apply suction?

- A.** No pre-oxygenation is needed, since his saturation is 94%.
- B.** Only while withdrawing the catheter, for no more than 10–15 seconds per pass.
- C.** While inserting and withdrawing, to clear more secretions.
- D.** Until no more secretions are returned, however long that takes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

What is the main reason each suction pass is limited to about 10–15 seconds?

- A.** Secretions are always cleared within 15 seconds.
- B.** It prevents infection from the catheter.
- C.** Longer passes remove oxygen and can cause hypoxia and bradycardia.
- D.** It keeps the catheter from wearing out.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

In an operating room that was renovated last year, the circulating nurse has checked the patient's ID band and consent in pre-op. The surgeon is ready to begin a left knee procedure. The patient is a retired dentist. What must happen before the incision?

- A.** Nothing more; the pre-op check already confirmed everything.
- B.** A time-out where the whole team confirms the patient, procedure, and site.
- C.** The time-out can be skipped if the surgeon marked the site.
- D.** The circulating nurse alone rechecks the consent.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

Two days after abdominal surgery, a 44-year-old librarian, whose husband went to get coffee, says she felt something “give” while coughing. The nurse sees loops of bowel through the opened incision. What should the nurse do?

- A.** Cover the wound with sterile saline-moistened dressings, place her in low Fowler's with knees bent, and notify the surgeon.
- B.** Gently push the bowel back inside and apply a dry dressing.
- C.** Cover with dry sterile gauze and wait for rounds.
- D.** Leave to find her husband so he can be with her.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A nurse cleaning a clean, closed abdominal incision for a patient who owns a hardware store and plays golf on weekends uses sterile swabs. The incision is 12 cm long. What is the correct technique?

- A.** Use one swab per 6 cm, so two swabs in all.
- B.** Start at the outer skin and work toward the incision.
- C.** Scrub back and forth with one swab.
- D.** Clean along the incision first, then move outward, using a new swab for each stroke.

Target word _____	The one fact that decides _____	Answer ____	☐ sure ☐ unsure ☐ guess
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12.

Right after birth, a newborn, whose parents are both teachers, has secretions in the mouth and nose and is crying. The room is 24 °C. Using a bulb syringe, what should the nurse do?

- A.** Suction only the nose, since babies breathe through it.
- B.** Suction the mouth first, then the nose.
- C.** Suction the nose first, then the mouth.
- D.** Suction deep into the throat until secretions stop.

Target word _____	The one fact that decides _____	Answer ____	☐ sure ☐ unsure ☐ guess
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STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A 67-year-old retired firefighter receiving IV fluids at 100 mL/hr through a forearm catheter, ~~placed two days ago by the night nurse~~, says the arm feels tight. The site is swollen, pale, and cool to touch, and the pump alarms for occlusion. What is happening?

Target word: identify complication **Decides:** Cool, pale, swollen = infiltration.

A	Camouflage	Uses the alarm detail and misses the site finding.
B	Absolute language	Swelling and coolness are never expected.
C	Correct	Fluid is leaking into the tissue.
D	Confusion set	Phlebitis is warm and red; also, the IV should not keep running.

2. Answer D Concept 1 • transfer

An IV site is red, warm, and tender, with a firm cord along the vein. Which complication is this?

Target word: reverse: from findings to name **Decides:** Warm, red, tender cord = phlebitis.

A	Dropped element	Drops the local site findings; overload shows in the lungs and vital signs.
B	Inverted operation	Infiltration is cool and pale.
C	Adjacent category	A systemic emergency with chest pain and breathlessness.
D	Correct	Inflammation of the vein wall.

3. Answer C Concept 2

On a rural unit where pumps are all in use, a patient ~~who works as a school custodian~~ needs 1,000 mL of normal saline over 8 hours. The tubing drop factor is 15 gtt/mL. ~~The bag was hung at 0800.~~ How many drops per minute?

Target word: gtt/min **Decides:** $(1,000 \times 15) \div 480$ min.

A	Inverted operation	Divides 125 mL/hr by the drop factor instead of multiplying.
B	Adjacent category	That is the mL/hr rate, not drops per minute.
C	Correct	$15,000 \div 480 = 31.25 \approx 31$.
D	Dropped element	Multiplies 125×15 but drops the division by 60 minutes.

4. Answer D Concept 2 • transfer

A gravity IV is running at 25 gtt/min through tubing with a drop factor of 10 gtt/mL. What is the hourly rate?

Target word: solve for mL/hr **Decides:** $\text{mL/hr} = \text{gtt/min} \times 60 \div \text{drop factor}$.

A	Wrong stem number	The drop rate used as mL/hr.
B	Inverted operation	Multiplies by the drop factor instead of dividing.
C	Dropped element	Divides by the drop factor but drops the 60 minutes.
D	Correct	$25 \times 60 = 1,500 \text{ gtt/hr} \div 10 = 150 \text{ mL/hr}$.

5. Answer A Concept 3

Ten minutes into a unit of packed red blood cells, a 58-year-old ~~bank manager, whose brother donated blood last week,~~ reports chills and low back pain. His temperature rose 1.5 °C. What should the nurse do first?

Target word: first action in a transfusion reaction **Decides:** Any reaction: stop the blood first.

A	Correct	Stopping limits how much incompatible blood enters.
B	Camouflage	Answers the donor detail and delays stopping.
C	Dropped element	Drops the rule that any reaction means stop.
D	Inverted operation	Treats the symptom while the cause keeps flowing.

6. Answer A Concept 3 • transfer

Which IV solution may be run through the same line as packed red blood cells?

Target word: reverse: compatible solution **Decides:** Only isotonic saline is compatible.

A	Correct	Normal saline does not damage red cells or cause clotting.
B	Absolute language	Only normal saline is approved.
C	Confusion set	Dextrose can make red cells clump and break down.
D	Adjacent category	Its calcium can trigger clotting in the tubing.

7. Answer B Concept 4

A patient with a tracheostomy, ~~who was a long-distance runner and communicates with a whiteboard,~~ needs suctioning. His oxygen saturation is 94%. How should the nurse apply suction?

Target word: suction technique **Decides:** Suction only on withdrawal, 10–15 seconds.

A	Camouflage	Uses the saturation detail; pre-oxygenation is still standard.
B	Correct	Limits oxygen loss and mucosal injury.
C	Inverted operation	Suction on insertion injures tissue and removes oxygen.
D	Absolute language	Long passes cause hypoxia.

8. Answer C Concept 4 • transfer

What is the main reason each suction pass is limited to about 10–15 seconds?

Target word: reverse: the reason behind the limit **Decides:** Suction removes air along with secretions.

A	Absolute language	Some patients need several passes; the limit is about oxygen.
B	Confusion set	Sterile technique prevents infection; time limits prevent hypoxia.
C	Correct	Suction pulls air out of the airway as well as mucus.
D	Adjacent category	Equipment life is not the reason.

9. Answer B Concept 5

In an operating room ~~that was renovated last year~~, the circulating nurse has checked the patient's ID band and consent in pre-op. The surgeon is ready to begin a left knee procedure. ~~The patient is a retired dentist.~~ What must happen before the incision?

Target word: step before incision **Decides:** Universal protocol time-out.

A	Dropped element	Drops the required time-out.
B	Correct	The pause is the final check just before incision.
C	Absolute language	Site marking and time-out are both required.
D	Adjacent category	The time-out involves the whole team, not one person.

10. Answer A Concept 6

Two days after abdominal surgery, a 44-year-old librarian, ~~whose husband went to get coffee~~, says she felt something "give" while coughing. The nurse sees loops of bowel through the opened incision. What should the nurse do?

Target word: action for evisceration **Decides:** Moist sterile cover, positioning, notify.

A	Correct	Moist cover protects tissue; positioning reduces strain.
B	Inverted operation	Never replace organs; dry gauze sticks to tissue.
C	Dropped element	Drops the moist dressing and urgent notification.
D	Camouflage	The nurse stays with the patient.

11. Answer D Concept 7

A nurse cleaning a clean, closed abdominal incision for a patient ~~who owns a hardware store and plays golf on weekends~~ uses sterile swabs. ~~The incision is 12 cm long.~~ What is the correct technique?

Target word: cleaning direction **Decides:** Least to most contaminated.

A	Wrong stem number	Uses the length detail; each stroke needs a new swab.
B	Inverted operation	Carries germs toward the cleanest area.
C	Absolute language	Reuses a contaminated swab.
D	Correct	The incision is the cleanest area; work outward.

12. Answer B Concept 8

Right after birth, a newborn, ~~whose parents are both teachers~~, has secretions in the mouth and nose and is crying. ~~The room is 24 °C.~~ Using a bulb syringe, what should the nurse do?

Target word: suction order **Decides:** Mouth before nose.

A	Dropped element	Leaves mouth secretions that can be aspirated.
B	Correct	Stimulating the nose can cause a gasp that pulls mouth secretions into the lungs.
C	Inverted operation	Reverses the safe order.
D	Adjacent category	Deep suction is not routine and can cause bradycardia.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	7	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	6	
Confusion set	Every option is real; the test is exact role-matching.	3	
Camouflage	Answers a stem fact that was never the question.	4	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Infiltration vs. phlebitis	Infiltration is IV fluid leaking into tissue: cool, pale, swollen, slow flow. Phlebitis is vein inflammation: warm, red, tender, sometimes a cord along the vein. Either way, stop the infusion and remove the catheter.	Mixing up warm and cool findings, or just slowing the rate.	A description of an IV site's color, temperature, and swelling.
2	Gravity drip rate	$\text{Drops per minute} = (\text{volume in mL} \times \text{drop factor in gtt/mL}) \div \text{time in minutes}$.	Stopping at mL/hr or dividing by the drop factor instead of multiplying.	A tubing drop factor (10, 15, 20, or 60 gtt/mL) and no pump.
3	Blood transfusion safety	Run blood only with 0.9% sodium chloride, stay with the patient for the first 15 minutes, and at any sign of reaction stop the transfusion first, then keep the vein open with saline through new tubing and notify.	Slowing the transfusion or giving a medication before stopping it.	Chills, fever, back pain, or itching during a transfusion.
4	Airway suctioning	Pre-oxygenate, insert without suction, apply suction only while withdrawing, and limit each pass to about 10-15 seconds.	Suctioning while inserting or for as long as secretions keep coming.	A suctioning procedure and a question about timing or technique.
5	Surgical time-out	Right before the incision, the whole team pauses to confirm the correct patient, procedure, and site; anyone can stop the process.	Thinking the pre-op check or a signed consent replaces the time-out.	The moment just before incision, or a team member with a doubt.
6	Wound dehiscence and evisceration	If organs protrude through a wound, stay with the patient, cover the wound with sterile saline-moistened dressings, put the patient in low Fowler's with knees bent, and notify the surgeon; never push organs back in.	Using dry gauze or trying to replace the organs.	A postoperative patient who feels something "give" after coughing.
7	Wound cleaning direction	Clean from the least contaminated area to the most — from the incision outward — using a new swab for each stroke.	Scrubbing back and forth or reusing one swab.	A wound-cleaning procedure and a direction choice.
8	Newborn bulb suctioning	Suction the mouth before the nose, because touching the nose can make the newborn gasp and inhale mouth secretions.	Suctioning the nose first because it looks more blocked.	A newborn with secretions and a bulb syringe.

Course scope (public description): Intermediate Nursing Skills will build on basic nursing skill concepts to develop intermediate medical surgical nursing practice, including peripheral intravenous access, blood

administration, airway management, perioperative and postoperative care, and wound care management. The course focuses on nursing care of both adult and pediatric populations, including a focus on the care of women and the maternal care setting. Students will use simulation to apply the clinical judgment model to various diverse populations in various care settings. Students are required to be successful on course performance assessments to progress to Adult Health II clinical. Co-requisites: D446 & D447